

SETROF TABLET

VISET16-var1 (MY)

DESCRIPTION

Oblong, white to off white film-coated tablet, shallow convex with break bar on one side and HD embossed on the other side.

COMPOSITION

Sertraline Hydrochloride 56 mg equivalent to Sertraline 50 mg/ tablet.

ACTIONS AND PHARMACOLOGY

Sertraline is a potent and selective inhibitor of neuronal uptake of serotonin (5-hydroxytryptamine [5-HT]). It has very weak effects on neuronal uptake of norepinephrine and dopamine. Chronic administration of sertraline in animals has resulted in down-regulation of postsynaptic beta-adrenergic receptors.

Sertraline lacks affinity for adrenergic (alpha1, alpha2, or beta) receptors, muscarinic – cholinergic receptors, gamma aminobutyric acid (GABA) receptors, dopaminergic receptors, histaminergic receptors, serotonergic (5-HT1A, 5-HT1B, 5-HT2) receptors, and benzodiazepine receptors. Sertraline does not inhibit monoamine oxidase.

PHARMACOKINETICS

Sertraline is slowly absorbed from the gastrointestinal tract with peak plasma concentrations occurring about 4.5 to 8.5 hours after ingestion. Elimination half time of sertraline is reported to be 24 to 26 hours. Both sertraline and its metabolites are extensively distributed into body tissues. About 98% is bound to plasma proteins. Sertraline undergoes extensive first-pass metabolism in the liver. The main pathway is demethylation to N-desmethylsertraline which is inactive. Both sertraline and N-desmethylsertraline undergo oxidative deamination and subsequent reduction, hydroxylation and glucuronide conjugation. About 40 to 45% of an administered dose is excreted in the urine and faeces in 9 days, with less than 0.2% recovered unchanged in urine and 12% to 14% unchanged sertraline in faeces.

INDICATIONS

For treatment of symptoms of depression including depression accompanied by symptoms of anxiety. For treatment of obsessions and compulsions in patients with obsessive-compulsive disorder (OCD). For treatment of panic disorder with or without agoraphobia.

CONTRAINDICATIONS

Concomitant use of monoamine oxidase inhibitors (MAOIs) with sertraline is contraindicated.

WARNING AND PRECAUTIONS

- Not taking sertraline with or within 14 days of taking an MAO inhibitor, not taking an MAO inhibitor within 14 days of taking sertraline.
- Avoiding use of alcohol beverages.
- Possible drowsiness, impairment of judgement, thinking, or motor skills

Risk benefits should be considered when the following medical problems exist:

- Hepatic function impairment
- History of mania
- Neurological impairment, including development delay
- Renal function impairment
- Seizure disorders
- Sensitivity to sertraline
- Weight loss

MAIN SIDE/ADVERSE EFFECTS

Gastrointestinal disorders: microscopic colitis / colitis microscopic*
*ADR identified post-marketing

In pediatric OCD patients, side-effects which occurred significantly more frequently with sertraline than placebo were: headache, insomnia, agitation, anorexia, tremor. Most were of mild to moderate severity.

Blood & lymphatic system disorders: thrombocytopenia*
*ADR identified post-marketing.

DRUG INTERACTIONS

- Concurrent use of MAO inhibitors with sertraline may result in hyperpyretic episodes, severe convulsions, hypertensive crises, or the serotonin syndrome; fatalities have occurred.
- Sertraline may inhibit the metabolism of tricyclic (TCAs).
- Concurrent use of terfenadine with sertraline may increase the risk of cardiac arrhythmias.
- Concurrent use of a highly protein-bound drug (e.g. digitoxin and warfarin) with sertraline may lead to increased plasma concentration of free medications and increased risk of adverse effects.
- Concurrent use of lithium with sertraline may lead to an increased incidence of serotonin-associated side effects.

PREGNANCY AND LACTATION

Pregnancy: Although animal studies did not provide any evidence of teratogenicity, the safety of Setrof Tablet during human pregnancy has not been established. As with all drugs Setrof Tablet should only be used in pregnancy if the potential benefits of treatment to the mother outweigh the possible risks to the developing foetus.

Observational data indicate an increased risk (less than 2-fold) of postpartum haemorrhage following SSRI/SNRI exposure within the month prior to birth.

Lactation: Setrof Tablet is known to be excreted in breast milk. Its effects on the nursing infant have not yet been established. If treatment with Setrof Tablet is considered necessary, discontinuation of breast feeding should be considered.

SYMPTOMS AND TREATMENT FOR OVERDOSE

Acute overdose of sertraline may develop the serotonin syndrome, anxiety, drowsiness, electrocardiogram (ECG) changes, mydriasis, nausea, tachycardia and vomiting.

Administering activated charcoal to decrease the absorption of sertraline.

DOSEAGE AND ADMINISTRATION

Adults

- **Depression (including accompanying symptoms of anxiety):** The starting dose is 50 mg daily and the usual antidepressant dose is 50 mg daily. In some patients, doses higher than 50 mg may be required.
- **Obsessive Compulsive Disorder:** The starting dose is 50 mg daily, and the therapeutic dose range is 50-200 mg daily.
- **Post-Traumatic Stress Disorder:** Treatment for PTSD should be initiated at 25 mg/day. After one week, the dose should be increased to 50 mg once daily. PTSD is a heterogeneous illness and some patient groups fulfilling the criteria for PTSD do not appear to be responsive to treatment with Setrof Tablet. Dosing should be reviewed periodically by the prescribing physician to determine response to therapy and treatment should be withdrawn if there is no clear evidence of efficacy.

Use in children aged 6 - 17 years

Treatment should only be initiated by specialists.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage: Store below 30°C.

Presentation/Packing: Blister pack of 3 x 10's, 10x10's.

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